Thyrocare

Tests you can trust

Name : Mathew Joseph(52Y/M)

Date : 13 Aug 2025

Test Asked : Package 4 At Home

Report Status: Complete Report

100% NABL ACCREDITATION

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98% Reports released within **06 Hours** of sample reaching the lab+

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PROCESSED AT :
Thyrocare,
5CA-711, 3rd Floor,
HRBR 2nd Block,
Hennur, Bengaluru-560043

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Thyrocare Technologies Limited, D-37/3, TTC MIDC, Turbhe, Navi Mumbai - 400 703 9870666333 wellness@thyrocare.com

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NAME : MATHEW JOSEPH(52Y/M)
REF. BY : SELF
TEST ASKED : PACKAGE 4 AT HOME

Home Collection
FLAT NUMBER D-2 FOURTH FLOOR
MALIACKAL,SHALOM PARADISE APTS,NO.30,1 A
MAINROAD,3RD A CROSS,NEW RAJANNA LAY

Report Availability Summary

Note: Please refer to the table below for status of your tests.

15 Ready **0** Ready with Cancellation **0** Processing **0** Cancelled in Lab

TEST DETAILS	REPORT STATUS
PACKAGE 4 AT HOME	Ready
ERYTHROCYTE SEDIMENTATION RATE (ESR)	Ready
HEMOGRAM - 6 PART (DIFF)	Ready
HbA1c	Ready
KIDPRO	Ready
IRON DEFICIENCY PROFILE	Ready
VITAMIN B-12	Ready
HIGH SENSITIVITY C-REACTIVE PROTEIN (HS-CRP)	Ready
LACTATE DEHYDROGENASE (LDH)	Ready
SERUM ELECTROLYTES	Ready
LIPID PROFILE	Ready
COMPLETE URINE ANALYSIS	Ready
PHOSPHOROUS	Ready
MAGNESIUM	Ready
LIVER FUNCTION TESTS	Ready
25-OH VITAMIN D (TOTAL)	Ready

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Summary Report

Tests outside reference range

TEST NAME	OBSERVED VALUE	UNITS	Bio. Ref. Interval.
COMPLETE HEMOGRAM			
LYMPHOCYTE	53	%	20-40
LYMPHOCYTES - ABSOLUTE COUNT	3.92	X 10^3 / µL	1.0-3.0
NEUTROPHILS	38	%	40-80
TOTAL RBC	5.6	X 10^6/µL	4.5-5.5
DIABETES			
HbA1c	5.8	%	< 5.7
LIPID			
HDL / LDL RATIO	0.33	Ratio	> 0.40
LDL CHOLESTEROL - DIRECT	147	mg/dL	< 100
NON-HDL CHOLESTEROL	162.1	mg/dL	< 160
TOTAL CHOLESTEROL	210	mg/dL	< 200
LIVER			
ALANINE TRANSAMINASE (SGPT)	83.1	U/L	< 45
ASPARTATE AMINOTRANSFERASE (SGOT)	47.3	U/L	< 35
RENAL			
URIC ACID	7.6	mg/dL	4.2 - 7.3
VITAMIN			
25-OH VITAMIN D (TOTAL)	21.1	ng/mL	30-100

Disclaimer: The above listed is the summary of the parameters with values outside the BRI. For detailed report values, parameter correlation and clinical interpretation, kindly refer to the same in subsequent pages.

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TEST NAME	TECHNOLOGY	VALUE	UNITS
HbA1c	**H.P.L.C**	**5.8**	**%**

Bio. Ref. Interval. :

As per ADA Guidelines	Guidance For Known Diabetics
Below 5.7% : Normal	Below 6.5% : Good Control
5.7% - 6.4% : Prediabetic	6.5% - 7% : Fair Control
>=6.5% : Diabetic	7.0% - 8% : Unsatisfactory Control
	>8% : Poor Control

Method : Fully Automated H.P.L.C method

AVERAGE BLOOD GLUCOSE (ABG)	CALCULATED	120	mg/dL

Bio. Ref. Interval. :

90 - 120 mg/dl : Good Control
121 - 150 mg/dl : Fair Control
151 - 180 mg/dl : Unsatisfactory Control
> 180 mg/dl : Poor Control

Method : Derived from HBA1c values

Please correlate with clinical conditions.

Sample Collected on (SCT) : 13 Aug 2025 08:29
Sample Received on (SRT) : 13 Aug 2025 11:01
Report Released on (RRT) : 13 Aug 2025 13:38
Sample Type : EDTA Whole Blood
Labcode : 1308040351/DF543
Barcode : EB729807

Dr Syeda Sumaiya MD(Path)

M. Anuswara

Dr.Anuswara MBBS,MD (Biochem)

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TEST NAME	TECHNOLOGY	VALUE	UNITS
ERYTHROCYTE SEDIMENTATION RATE (ESR)	MODIFIED WESTERGREN	2	mm / hr

Bio. Ref. Interval. :-

Male : 0-15 Female : 0-20

Clinical Significance:
- An erythrocyte sedimentation rate (ESR) is a blood test that can rise if you have inflammation in your body. Its also used as a marker to monitor prognosis of an existing inflammatory/infective condition.
- Inflammation is your immune systems response to injury, infection, and many types of conditions, including immune system disorders, certain cancers and blood disorders.
- A high ESR test result may be from a condition that causes inflammation, such as: Arteritis, Arthritis, Systemic vasculitis, Polymyalgia rheumatica, Inflammatory bowel disease, Kidney disease, Infections like Tuberculosis etc, Rheumatoid arthritis and other autoimmune diseases, Heart disease, Certain cancers and many other Conditions.
- A low ESR test result may be caused by conditions such as: A blood disorder, such as: Polycythemia, Sickle cell disease (SCD), Leukocytosis, Heart failure, Certain kidney and liver problems etc.
- Certain physiological conditions also affect ESR results, these include : Pregnancy, menstrual cycle, ageing, obesity, drinking alcohol regularly, and exercise, Certain medicines and supplements also can affect ESR results.
- Hence Its always suggested to interpret ESR results in conjunction with Clinical History and other findings.

References :
https://medlineplus.gov/lab-tests/erythrocyte-sedimentation-rate-esr/

Please correlate with clinical conditions.
Method:- MODIFIED WESTERGREN

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MAINROAD,3RD A CROSS,NEW RAJANNA LAY

TEST NAME	METHODOLOGY	VALUE	UNITS	Bio. Ref. Interval.
HEMOGLOBIN	SLS-Hemoglobin Method	16.3	g/dL	13.0-17.0
Hematocrit (PCV)	CPH Detection	50	%	40.0-50.0
Total RBC	**HF & EI**	**5.6**	**X 10^6/µL**	**4.5-5.5**
Mean Corpuscular Volume (MCV)	Calculated	89.3	fL	83.0-101.0
Mean Corpuscular Hemoglobin (MCH)	Calculated	29.1	pq	27.0-32.0
Mean Corp.Hemo. Conc (MCHC)	Calculated	32.6	g/dL	31.5-34.5
Red Cell Distribution Width - SD (RDW-SD)	Calculated	41.3	fL	39-46
Red Cell Distribution Width (RDW - CV)	Calculated	12.6	%	11.6-14
RED CELL DISTRIBUTION WIDTH INDEX (RDWI)	Calculated	200.9	-	*Refer Note below
MENTZER INDEX	Calculated	15.9	-	*Refer Note below
TOTAL LEUCOCYTE COUNT (WBC)	HF & FC	7.39	X 10^3 / µL	4.0 - 10.0
DIFFERENTIAL LEUCOCYTE COUNT				
Neutrophils Percentage	**Flow Cytometry**	**38**	**%**	**40-80**
Lymphocytes Percentage	**Flow Cytometry**	**53**	**%**	**20-40**
Monocytes Percentage	Flow Cytometry	3	%	2-10
Eosinophils Percentage	Flow Cytometry	5.4	%	1-6
Basophils Percentage	Flow Cytometry	0.3	%	0-2
Immature Granulocyte Percentage (IG%)	Flow Cytometry	0.3	%	0-0.5
Nucleated Red Blood Cells %	Flow Cytometry	0.01	%	0.0-5.0
ABSOLUTE LEUCOCYTE COUNT				
Neutrophils - Absolute Count	Calculated	2.81	X 10^3 / µL	2.0-7.0
Lymphocytes - Absolute Count	**Calculated**	**3.92**	**X 10^3 / µL**	**1.0-3.0**
Monocytes - Absolute Count	Calculated	0.22	X 10^3 / µL	0.2 - 1.0
Basophils - Absolute Count	Calculated	0.02	X 10^3 / µL	0.02 - 0.1
Eosinophils - Absolute Count	Calculated	0.4	X 10^3 / µL	0.02 - 0.5
Immature Granulocytes (IG)	Calculated	0.02	X 10^3 / µL	0-0.3
Nucleated Red Blood Cells	Calculated	0.01	X 10^3 / µL	0.0-0.5
PLATELET COUNT	HF & EI	284	X 10^3 / µL	150-410
Mean Platelet Volume (MPV)	Calculated	9.5	fL	6.5-12
Platelet Distribution Width (PDW)	Calculated	10.1	fL	9.6-15.2
Platelet to Large Cell Ratio (PLCR)	Calculated	20.6	%	19.7-42.4
Plateletcrit (PCT)	Calculated	0.27	%	0.19-0.39

Remarks : Alert!!! Predominantly normocytic normochromic with ovalocytes. Platelets:Appear adequate in smear.

***Note - Mentzer index (MI), RDW-CV and RDWI are hematological indices to differentiate between Iron Deficiency Anemia (IDA) and Beta Thalassemia Trait (BTT). MI >13, RDWI >220 and RDW-CV >14 more likely to be IDA. MI <13, RDWI <220, and RDW-CV <14 more likely to be BTT. Suggested Clinical correlation. BTT to be confirmed with HB electrophoresis if clinically indicated.**

Method : Fully automated bidirectional analyser (6 Part Differential SYSMEX XN-1000)

(Reference : *FC- flowcytometry, *HF- hydrodynamic focussing, *EI- Electric Impedence, *Hb- hemoglobin, *CPH- Cumulative pulse height**)**

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TEST NAME	TECHNOLOGY	VALUE	UNITS
25-OH VITAMIN D (TOTAL)	**E.C.L.I.A**	**21.1**	**ng/mL**

Bio. Ref. Interval. :-

Deficiency : <=20 ng/ml || Insufficiency : 21-29 ng/ml
Sufficiency : >= 30 ng/ml || Toxicity : >100 ng/ml

Clinical Significance:
Vitamin D is a fat soluble vitamin that has been known to help the body absorb and retain calcium and phosphorous; both are critical for building bone health.
Decrease in vitamin D total levels indicate inadequate exposure of sunlight, dietary deficiency, nephrotic syndrome.
Increase in vitamin D total levels indicate Vitamin D intoxication.

Specifications: Precision: Intra assay (%CV):9.20%, Inter assay (%CV):8.50%
Kit Validation Reference : Holick M. Vtamin D the underappreciated D-Lightful hormone that is important for Skeletal and cellular health Curr Opin Endocrinol Diabetes 2002:9(1)87-98.

Please correlate with clinical conditions.
Method:- Fully Automated Electrochemiluminescence Compititive Immunoassay

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TEST NAME	TECHNOLOGY	VALUE	UNITS
VITAMIN B-12	E.C.L.I.A	375	pg/mL

Bio. Ref. Interval. :-

Normal: 197-771 pg/ml

Clinical significance :
Vitamin B12 or cyanocobalamin, is a complex corrinoid compound found exclusively from animal dietary sources, such as meat, eggs and milk. It is critical in normal DNA synthesis, which in turn affects erythrocyte maturation and in the formation of myelin sheath. Vitamin-B12 is used to find out neurological abnormalities and impaired DNA synthesis associated with macrocytic anemias. For diagnostic purpose, results should always be assessed in conjunction with the patients medical history, clinical examination and other findings.

Specifications: Intra assay (%CV):2.6%, Inter assay (%CV):2.3 %

Kit Validation Reference : Thomas L.Clinical laborator Diagnostics : Use and Assessment of Clinical laboratory Results 1st Edition,TH Books-Verl-Ges,1998:424-431

Please correlate with clinical conditions.
Method:- Fully Automated Electrochemiluminescence Compititive Immunoassay

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TEST NAME	TECHNOLOGY	VALUE	UNITS
HIGH SENSITIVITY C-REACTIVE PROTEIN (HS-CRP)	IMMUNOTURBIDIMETRY	2.4	mg/L

Bio. Ref. Interval. :-

< 1.00 - Low Risk
1.00 - 3.00 - Average Risk
>3.00 - 10.00 - High Risk
> 10.00 - Possibly due to Non-Cardiac Inflammation

Disclaimer: Persistent unexplained elevation of HSCRP >10 should be evaluated for non-cardiovascular etiologies such as infection , active arthritis or concurrent illness.

Clinical significance:
High sensitivity C- reactive Protein (HSCRP) can be used as an independent risk marker for the identification of Individuals at risk for future cardiovascular Disease. A coronary artery disease risk assessment should be based on the average of two hs-CRP tests, ideally taken two weeks apart.

Kit Validation Reference:
1.Clinical management of laboratory date in medical practice 2003-3004, 207(2003).
2.Tietz : Textbook of Clinical Chemistry and Molecular diagnostics :Second edition :Chapter 47:Page no.1507- 1508.

Please correlate with clinical conditions.
Method:- FULLY AUTOMATED LATEX AGGLUTINATION – BECKMAN COULTER

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TEST NAME	TECHNOLOGY	VALUE	UNITS
IRON **Bio. Ref. Interval. :** Male : 65 - 175 Female : 50 - 170 **Method :** Ferrozine method without deproteinization	PHOTOMETRY	136	µg/dL
TOTAL IRON BINDING CAPACITY (TIBC) **Bio. Ref. Interval. :** Male: 225 - 535 µg/dl Female: 215 - 535 µg/dl **Method :** Spectrophotometric Assay	PHOTOMETRY	368	µg/dL
% TRANSFERRIN SATURATION **Bio. Ref. Interval. :** 13 - 45 **Method :** Derived from IRON and TIBC values	CALCULATED	37	%
UNSAT.IRON-BINDING CAPACITY(UIBC) **Bio. Ref. Interval. :** 162 - 368 **Method :** SPECTROPHOTOMETRIC ASSAY	PHOTOMETRY	231.79	µg/dL

Please correlate with clinical conditions.

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TEST NAME	TECHNOLOGY	VALUE	UNITS	Bio. Ref. Interval.
TOTAL CHOLESTEROL	**PHOTOMETRY**	**210**	**mg/dL**	**< 200**
HDL CHOLESTEROL - DIRECT	PHOTOMETRY	48	mg/dL	40-60
LDL CHOLESTEROL - DIRECT	**PHOTOMETRY**	**147**	**mg/dL**	**< 100**
TRIGLYCERIDES	PHOTOMETRY	102	mg/dL	< 150
TC/ HDL CHOLESTEROL RATIO	CALCULATED	4.4	Ratio	3 - 5
TRIG / HDL RATIO	CALCULATED	2.14	Ratio	< 3.12
LDL / HDL RATIO	CALCULATED	3.1	Ratio	1.5-3.5
HDL / LDL RATIO	**CALCULATED**	**0.33**	**Ratio**	**> 0.40**
NON-HDL CHOLESTEROL	**CALCULATED**	**162.1**	**mg/dL**	**< 160**
VLDL CHOLESTEROL	CALCULATED	20.4	mg/dL	5 - 40

Please correlate with clinical conditions.

Method :

CHOL - Cholesterol Oxidase, Esterase, Peroxidase
HCHO - Direct Enzymatic Colorimetric
LDL - Direct Measure
TRIG - Enzymatic, End Point
TC/H - Derived from serum Cholesterol and Hdl values
TRI/H - Derived from TRIG and HDL Values
LDL/ - Derived from serum HDL and LDL Values
HD/LD - Derived from HDL and LDL values.
NHDL - Derived from serum Cholesterol and HDL values
VLDL - Derived from serum Triglyceride values

***REFERENCE RANGES AS PER NCEP ATP III GUIDELINES:**

TOTAL CHOLESTEROL	(mg/dl)	HDL	(mg/dl)	LDL	(mg/dl)	TRIGLYCERIDES	(mg/dl)
DESIRABLE	<200	LOW	<40	OPTIMAL	<100	NORMAL	<150
BORDERLINE HIGH	200-239	HIGH	>60	NEAR OPTIMAL	100-129	BORDERLINE HIGH	150-199
HIGH	>240			BORDERLINE HIGH	130-159	HIGH	200-499
				HIGH	160-189	VERY HIGH	>500
				VERY HIGH	>190		

Alert !!! 10-12 hours fasting is mandatory for lipid parameters. If not, values might fluctuate.

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TEST NAME	TECHNOLOGY	VALUE	UNITS	Bio. Ref. Interval.
ALKALINE PHOSPHATASE	PHOTOMETRY	80.82	U/L	45-129
BILIRUBIN - TOTAL	PHOTOMETRY	1	mg/dL	0.3-1.2
BILIRUBIN -DIRECT	PHOTOMETRY	0.2	mg/dL	< 0.3
BILIRUBIN (INDIRECT)	CALCULATED	0.8	mg/dL	0-0.9
GAMMA GLUTAMYL TRANSFERASE (GGT)	PHOTOMETRY	29.5	U/L	< 55
ASPARTATE AMINOTRANSFERASE (SGOT)	**PHOTOMETRY**	**47.3**	**U/L**	**< 35**
ALANINE TRANSAMINASE (SGPT)	**PHOTOMETRY**	**83.1**	**U/L**	**< 45**
SGOT / SGPT RATIO	CALCULATED	0.57	Ratio	< 2
PROTEIN - TOTAL	PHOTOMETRY	7.44	gm/dL	5.7-8.2
ALBUMIN - SERUM	PHOTOMETRY	4.13	gm/dL	3.2-4.8
SERUM GLOBULIN	CALCULATED	3.31	gm/dL	2.5-3.4
SERUM ALB/GLOBULIN RATIO	CALCULATED	1.25	Ratio	0.9 - 2

Please correlate with clinical conditions.

Method :

ALKP - Modified IFCC method
BILT - Vanadate Oxidation
BILD - Vanadate Oxidation
BILI - Derived from serum Total and Direct Bilirubin values
GGT - Modified IFCC method
SGOT - IFCC* Without Pyridoxal Phosphate Activation
SGPT - IFCC* Without Pyridoxal Phosphate Activation
OT/PT - Derived from SGOT and SGPT values.
PROT - Biuret Method
SALB - Albumin Bcg¹method (Colorimetric Assay Endpoint)
SEGB - DERIVED FROM SERUM ALBUMIN AND PROTEIN VALUES
A/GR - Derived from serum Albumin and Protein values

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TEST NAME	TECHNOLOGY	VALUE	UNITS
MAGNESIUM	PHOTOMETRY	2.01	mg/dL

Bio. Ref. Interval. :-

1.90 - 3.10 mg/dL

Clinical significance:

Magnesium is the fourth most abundant cation in the body and second most prevalent intracellular cation. The total body magnesium content is about 25 g or approximately 1 mol, of which 55% reside in the skeleton. About 45% of the magnesium is intracellular. In general higher the metabolic activity of cell, the greater is its magnesium content. Magnesium is a cofactor for more than 300 enzymes in the body.

Disorders of magnesium metabolism are separated into those causing hypomagnesaemia/magnesium deficiencies and hypermagnesemia. Hypomagnesaemia is common in patient in hospitals. Moderate to severe deficiency of magnesium is usually due to loss of magnesium from the gastrointestinal (gi) tract or kidneys. One of the more serious complications of magnesium deficiency is cardiac arrhythmia. Symptomatic hypermagnsemia is almost always caused by excessive intake, resulting from administration of antacids, enemas, and parenteral fluids containing magnesium. Depression of neuromuscular system is the most common manifestation of magnesium intoxication.

External quality control program participation:

College Of American Pathologists: Chemistry survey; CAP Number: 7193855-01

Please correlate with clinical conditions.
Method:- MODIFIED XYLIDYL BLUE REACTION METHOD

Sample Collected on (SCT) : 13 Aug 2025 08:29
Sample Received on (SRT) : 13 Aug 2025 10:27
Report Released on (RRT) : 13 Aug 2025 13:22
Sample Type : SERUM
Labcode : 1308066862/DF543
Barcode : DX270603

Dr Syeda Sumaiya MD(Path)

M. Anuswara

Dr.Anuswara MBBS,MD (Biochem)

NAME : MATHEW JOSEPH(52Y/M)
REF. BY : SELF
TEST ASKED : PACKAGE 4 AT HOME

Home Collection
FLAT NUMBER D-2 FOURTH FLOOR MALIACKAL,SHALOM PARADISE APTS,NO.30,1 A MAINROAD,3RD A CROSS,NEW RAJANNA LAY OUT,HORMAVU,AGRA BANGALORE,560043 NO.30,1 A MAINROAD,3RD A CROSS,NEW RAJANNA LAY OUT

TEST NAME	TECHNOLOGY	VALUE	UNITS
LACTATE DEHYDROGENASE (LDH)	PHOTOMETRY	147.1	U/L

Bio. Ref. Interval. :-

120-250

Clinical Significance:
Lactate Dehydrogenase occurs in the cytoplasm of all cells;there are five isoenzymes. The highest concentration are found in heart,liver,skeletal muscle,kidney, and the RBCs,with lesser amounts in lung,smooth muscle, and brain . LD catalyzes the interconversion of lactate and pyruvate. Marked elevations in lactate dehydrogenase (LD) activity can be observed in megaloblastic anemia, untreated pernicious anemia, Hodgkins disease, abdominal and lung cancers, severe shock, and hypoxia. Moderate to slight increases in LD levels are seen in myocardial infarction (MI), pulmonary infarction, pulmonary embolism, leukemia, hemolytic anemia, infectious mononucleosis, progressive muscular dystrophy (especially in the early and middle stages of the disease), liver disease, and renal disease. In liver disease, elevations of LD are not as great as the increases in aspartate amino transferase (AST) and alanine aminotransferase (ALT). Increased levels of the enzyme are found in about one third of patients with renal disease, especially those with tubular necrosis or pyelonephritis. However, these elevations do not correlate well with proteinuria or other parameters of renal disease. On occasion a raised LD level may be the only evidence to suggest the presence of a hidden pulmonary embolus.

Specifications: Precision: Intra assay (%CV): 0.76, Inter assay (%CV): 1.8, Sensitivity: >=0.01

Kit validation references:

Amador E. Dorfman L. E. Wacker W. E., Clin. Chem., 1963; 331

Please correlate with clinical conditions.
Method:- LACTATE / NAD METHOD

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Dr.Anuswara MBBS,MD (Biochem)

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TEST NAME	TECHNOLOGY	VALUE	UNITS
PHOSPHOROUS	PHOTOMETRY	3.9	mg/dL

Bio. Ref. Interval. :
Adults : 2.4 - 5.1 mg/dL
Children : 4.0 - 7.0 mg/dL

Clinical Significance:
In plasma and serum the majority of phosphate exists in the inorganic form (Pi), approximately 15% bound to protein and the remainder in complexes and free forms. Serum phosphate concentrations are dependent on diet and variation in the secretion of hormones such as Parathyroid Hormone (PTH).

Specifications:
Precision %CV :- Intra assay %CV- 1.55% , Inter assay %CV-2.99% , Sensitivity:-0.10 mmol/L

Kit Validation Reference:
Young DS. Effects of drugs on clinical laboratory tests, 5th ed. AACC Press, 2000.
Method : UNREDUCED PHOSPHOMOLYBDATE METHOD

Please correlate with clinical conditions.

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TEST NAME	TECHNOLOGY	VALUE	UNITS
SODIUM	I.S.E - INDIRECT	137.38	mmol/L

Bio. Ref. Interval. :
Adults: 136-145 mmol/l
Method : ION SELECTIVE ELECTRODE - INDIRECT

POTASSIUM	I.S.E - INDIRECT	4.51	mmol/L

Bio. Ref. Interval. :
ADULTS: 3.5-5.1 MMOL/L

Clinical Significance :
An abnormal increase in potassium (hyperkalemia)can profoundly affect the nervous system and increase the chance of irregular heartbeats (arrhythmias), which ,when extreme ,can be fatal. The assay could be affected mildly and may result in anomalous values if serum samples have heterophilic antibodies, hemolyzed , icteric or lipemic. The concentration of Potassium in a given specimen may vary due to differences in assay methods, calibration and reagent specificity.
Method : ION SELECTIVE ELECTRODE - INDIRECT

CHLORIDE	I.S.E - INDIRECT	101.3	mmol/L

Bio. Ref. Interval. :
ADULTS: 98-107 MMOL/L

Clinical Significance :
An increased level of blood chloride (called hyperchloremia) usually indicates dehydration, but can also occur with other problems that cause high blood sodium, such as Cushing syndrome or kidney disease. Hyperchloremia also occurs when too much base is lost from the body (producing metabolic acidosis) or when a person hyperventilates (causing respiratory alkalosis). A decreased level of blood chloride (called hypochloremia) occurs with any disorder that causes low blood sodium. Hypochloremia also occurs with congestive heart failure, prolonged vomiting or gastric suction, Addison disease, emphysema or other chronic lung diseases (causing respiratory acidosis), and with loss of acid from the body (called metabolic alkalosis).
Method : ION SELECTIVE ELECTRODE - INDIRECT

Please correlate with clinical conditions.

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Barcode : DX270603

Dr Syeda Sumaiya MD(Path)

M. Anuswara

Dr.Anuswara MBBS,MD (Biochem)

PROCESSED AT :
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HRBR 2nd Block,
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NAME : MATHEW JOSEPH(52Y/M)
REF. BY : SELF
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TEST NAME	TECHNOLOGY	VALUE	UNITS	Bio. Ref. Interval.
BLOOD UREA NITROGEN (BUN)	PHOTOMETRY	9.4	mg/dL	7.94 - 20.07
CREATININE - SERUM	PHOTOMETRY	0.85	mg/dL	0.72-1.18
BUN / SR.CREATININE RATIO	CALCULATED	11.06	Ratio	9:1-23:1
UREA (CALCULATED)	CALCULATED	20.12	mg/dL	Adult : 17-43
UREA / SR.CREATININE RATIO	CALCULATED	23.67	Ratio	< 52
CALCIUM	PHOTOMETRY	9.5	mg/dL	8.8-10.6
URIC ACID	**PHOTOMETRY**	**7.6**	**mg/dL**	**4.2 - 7.3**

Please correlate with clinical conditions.

Method :

BUN - Kinetic UV Assay.
SCRE - Creatinine Enzymatic Method
B/CR - Derived from serum Bun and Creatinine values
UREAC - Derived from BUN Value.
UR/CR - Derived from UREA and Sr.Creatinine values.
CALC - Arsenazo III Method, End Point.
URIC - Uricase / Peroxidase Method

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Dr Syeda Sumaiya MD(Path)

M. Anuswara

Dr.Anuswara MBBS,MD (Biochem

NAME : MATHEW JOSEPH(52Y/M)
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TEST NAME	TECHNOLOGY	VALUE	UNITS
EST. GLOMERULAR FILTRATION RATE (eGFR)	CALCULATED	105	mL/min/1.73 m2

Bio. Ref. Interval. :-

> = 90 : Normal
60 - 89 : Mild Decrease
45 - 59 : Mild to Moderate Decrease
30 - 44 : Moderate to Severe Decrease
15 - 29 : Severe Decrease

Clinical Significance

The normal serum creatinine reference interval does not necessarily reflect a normal GFR for a patient. Because mild and moderate kidney injury is poorly inferred from serum creatinine alone. Thus, it is recommended for clinical laboratories to routinely estimate glomerular filtration rate (eGFR), a "gold standard" measurement for assessment of renal function, and report the value when serum creatinine is measured for patients 18 and older, when appropriate and feasible. It cannot be measured easily in clinical practice, instead, GFR is estimated from equations using serum creatinine, age, race and sex. This provides easy to interpret information for the doctor and patient on the degree of renal impairment since it approximately equates to the percentage of kidney function remaining. Application of CKD-EPI equation together with the other diagnostic tools in renal medicine will further improve the detection and management of patients with CKD.

Reference

Levey AS, Stevens LA, Schmid CH, Zhang YL, Castro AF, 3rd, Feldman HI, et al. A new equation to estimate glomerular filtration rate. Ann Intern Med. 2009;150(9):604-12.

Please correlate with clinical conditions.
Method:- 2021 CKD EPI Creatinine Equation

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Dr Syeda Sumaiya MD(Path)

NAME : MATHEW JOSEPH(52Y/M)
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TEST NAME	METHODOLOGY	VALUE	UNITS	Bio. Ref. Interval.
Complete Urinogram				
Physical Examination				
VOLUME	Visual Determination	3	mL	-
COLOUR	Visual Determination	PALE YELLOW	-	Pale Yellow
APPEARANCE	Visual Determination	CLEAR	-	Clear
SPECIFIC GRAVITY	pKa change	1.01	-	1.003-1.030
PH	pH indicator	6.5	-	5-8
Chemical Examination				
URINARY PROTEIN	PEI	ABSENT	mg/dL	Absent
URINARY GLUCOSE	GOD-POD	ABSENT	mg/dL	Absent
URINE KETONE	Nitroprusside	ABSENT	mg/dL	Absent
URINARY BILIRUBIN	Diazo coupling	ABSENT	mg/dL	Absent
UROBILINOGEN	Diazo coupling	Normal	mg/dL	<=0.2
BILE SALT	Hays sulphur	ABSENT	-	Absent
BILE PIGMENT	Ehrlich reaction	ABSENT	-	Absent
URINE BLOOD	Peroxidase reaction	ABSENT	-	Absent
NITRITE	Diazo coupling	ABSENT	-	Absent
LEUCOCYTE ESTERASE	Esterase reaction	ABSENT	-	Absent
Microscopic Examination				
MUCUS	Microscopy	ABSENT	-	Absent
RED BLOOD CELLS	Microscopy	ABSENT	cells/HPF	0-5
URINARY LEUCOCYTES (PUS CELLS)	Microscopy	ABSENT	cells/HPF	0-5
EPITHELIAL CELLS	Microscopy	ABSENT	cells/HPF	0-5
CASTS	Microscopy	ABSENT	-	Absent
CRYSTALS	Microscopy	ABSENT	-	Absent
BACTERIA	Microscopy	ABSENT	-	Absent
YEAST	Microscopy	ABSENT	-	Absent
PARASITE	Microscopy	ABSENT	-	Absent

(Reference : *PEI - Protein error of indicator, *GOD-POD - Glucose oxidase-peroxidase**)**

~~ End of report ~~

Sample Collected on (SCT) : 13 Aug 2025 08:29
Sample Received on (SRT) : 13 Aug 2025 11:10
Report Released on (RRT) : 13 Aug 2025 13:07
Sample Type : URINE
Labcode : 1308068423/DF543
Barcode : DQ005040

Dr Syeda Sumaiya MD(Path)

Scan QR code to verify authenticity of reported results; active for 30 days from release time.

CONDITIONS OF REPORTING

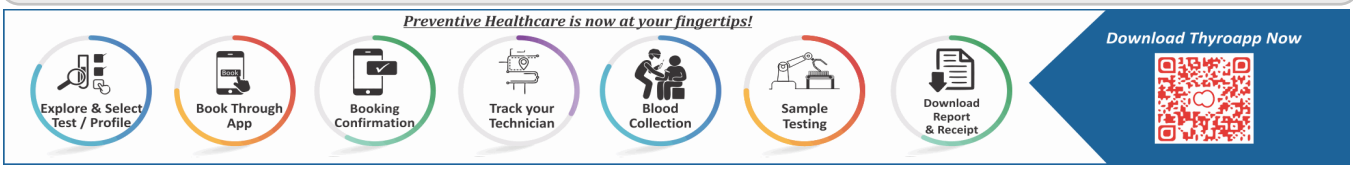
- The reported results are for information and interpretation of the referring doctor only.
- It is presumed that the tests performed on the specimen belong to the patient; named or identified.
- Results of tests may vary from laboratory to laboratory and also in some parameters from time to time for the same patient.
- Should the results indicate an unexpected abnormality, the same should be reconfirmed.
- Only such medical professionals who understand reporting units, reference ranges and limitations of technologies should interpret results.
- This report is not valid for medico-legal purpose.
- Neither Thyrocare, nor its employees/representatives assume: (a) any liability, responsibility for any loss or damage that may be incurred by any person as a result of presuming the meaning or contents of the report, (b) any claims of any nature whatsoever arising from or relating to the performance of the requested tests as well as any claim for indirect, incidental or consequential damages. The total liability, in any case, of Thyrocare shall not exceed the total amount of invoice for the services provided and paid for.
- Thyrocare Discovery video link :- https://youtu.be/nbdYeRgYyQc

EXPLANATIONS

- Majority of the specimen processed in the laboratory are collected by Pathologists and Hospitals we call them as "Clients".
- **Name** - The name is as declared by the client and recored by the personnel who collected the specimen.
- **Ref.Dr** - The name of the doctor who has recommended testing as declared by the client.
- **Labcode** - This is the accession number in our laboratory and it helps us in archiving and retrieving the data.
- **Barcode** - This is the specimen identity number and it states that the results are for the specimen bearing the barcode (irrespective of the name).
- **SCP** - Specimen Collection Point - This is the location where the blood or specimen was collected as declared by the client.
- **SCT** - Specimen Collection Time - The time when specimen was collected as declared by the client.
- **SRT** - Specimen Receiving Time - This time when the specimen reached our laboratory.
- **RRT** - Report Releasing Time - The time when our pathologist has released the values for Reporting.
- **Reference Range** - Means the range of values in which 95% of the normal population would fall.

SUGGESTIONS

- Values out of reference range requires reconfirmation before starting any medical treatment.
- Retesting is needed if you suspect any quality shortcomings.
- Testing or retesting should be done in accredited laboratories.
- For suggestions, complaints, clinical support or feedback, write to us at **customersupport@thyrocare.com** or call us on **022-3090 0000**



+ *T&C Apply, #As on 5th December 2024 (Applicable for all company owned labs except Bhagalpur & Vijayawada),*
* *As per survey on doctors' perception of laboratory diagnostics (IJARIIT, 2023), -Mumbai Reference Lab is CAP Accredited*